

Question 1

NOT COMPLETE

Case Study Screen 1 of 6

The following 6 questions refer to this clinical scenario: NURS 1021 Unit 6 Case Study 1.

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Click to highlight the findings below that would require follow-up.

Nurses' Notes

Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

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CORRECT

Score: 3 / 3 points

Rationale:

Loss of appetite is highlighted because it is a classic early symptom of appendicitis and other acute intra-abdominal inflammatory processes. The abdominal pain description is highlighted because pain rated 7/10, present for a full week, and attributed by the client to a single kick 9 days earlier is disproportionate to a single minor blunt-trauma event and instead suggests an evolving intra-abdominal process such as appendicitis, bowel obstruction, or an unrecognized traumatic injury (for example, to the spleen). The full vital sign set — T 103.4°F (39.7°C), P 92, RR 22, BP 130/86, and pulse oximetry 98% — is highlighted as one finding because the fever signals an infectious or inflammatory process, and together with the mildly elevated pulse and respiratory rate it reflects an early systemic response to that process, even though the pulse, respiratory rate, and oxygen saturation are each close to normal in isolation. Nausea, the narrative mention of vomiting/fever/constipation, and the client playing soccer with the child once a week restate or add context to the more specific findings above without independently requiring follow-up. No significant past medical or surgical history, the BMI of 32, and the client's social alcohol and cigarette use are baseline/lifestyle information, not acute findings, and do not require follow-up.

Question 2

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

**1100:** Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

➤ For each assessment finding below, click to specify if the finding is consistent with the disease process of bowel obstruction, appendicitis, or ruptured spleen. Each finding may support more than 1 disease process.

| Assessment Finding        | Bowel Obstruction        | Appendicitis             | Ruptured Spleen          |
|---------------------------|--------------------------|--------------------------|--------------------------|
| appetite                  | <input type="checkbox"/> | <input type="checkbox"/> | <input type="checkbox"/> |
| pain level                | <input type="checkbox"/> | <input type="checkbox"/> | <input type="checkbox"/> |
| bowel pattern             | <input type="checkbox"/> | <input type="checkbox"/> | <input type="checkbox"/> |
| gastrointestinal symptoms | <input type="checkbox"/> | <input type="checkbox"/> | <input type="checkbox"/> |

Note: Each column must have at least 1 response option selected.

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Question 2

COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

1100: Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

➤ For each assessment finding below, click to specify if the finding is consistent with the disease process of bowel obstruction, appendicitis, or ruptured spleen. Each finding may support more than 1 disease process.

| Assessment Finding        | Bowel Obstruction                   | Appendicitis                        | Ruptured Spleen                     |
|---------------------------|-------------------------------------|-------------------------------------|-------------------------------------|
| appetite                  | <input checked="" type="checkbox"/> | <input checked="" type="checkbox"/> | <input type="checkbox"/>            |
| pain level                | <input checked="" type="checkbox"/> | <input checked="" type="checkbox"/> | <input checked="" type="checkbox"/> |
| bowel pattern             | <input checked="" type="checkbox"/> | <input checked="" type="checkbox"/> | <input type="checkbox"/>            |
| gastrointestinal symptoms | <input checked="" type="checkbox"/> | <input checked="" type="checkbox"/> | <input type="checkbox"/>            |

Note: Each column must have at least 1 response option selected.

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CORRECT

Score: 9 / 9 points

Rationale:

Decreased appetite is consistent with bowel obstruction and appendicitis (both cause visceral inflammation or distension that suppresses appetite) but is not a typical finding of an isolated ruptured spleen, so appetite is checked under Bowel Obstruction and Appendicitis only. Pain level is checked under all 3 disease processes because bowel obstruction, appendicitis, and a ruptured spleen can each cause significant abdominal pain — obstruction from bowel wall distension, appendicitis from localized peritoneal irritation, and splenic rupture from hemoperitoneum and capsular stretch. Bowel pattern (constipation) is checked under Bowel Obstruction and Appendicitis, since mechanical obstruction directly halts stool passage and appendicitis commonly causes an associated ileus with constipation, while an isolated splenic injury does not typically alter bowel pattern. Gastrointestinal symptoms such as nausea and vomiting are checked under Bowel Obstruction and Appendicitis, both of which commonly trigger vomiting through bowel distension or peritoneal irritation, whereas an isolated splenic rupture (a vascular/hemorrhagic injury) does not typically produce prominent nausea and vomiting on its own.

Question 3

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

**1100:** Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

Select the 3 complications the client is at risk for developing.

- ☐ 1. anemia
- ☐ 2. peritonitis
- ☐ 3. septic shock
- ☐ 4. hypovolemia
- ☐ 5. dysrhythmias
- ☐ 6. cardiac arrest

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Question 3

COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

**1100:** Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

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- ☐ 5. dysrhythmias
- ☐ 6. cardiac arrest

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CORRECT

Score: 3 / 3 points

Rationale:

The client's presentation and CT findings (revealed later in the case) are consistent with a gangrenous, ruptured appendix. A ruptured appendix spills infected intestinal contents into the peritoneal cavity, so peritonitis (Option 2) — inflammation of the peritoneum — is a direct and expected complication. Untreated peritonitis can progress to septic shock (Option 3) as bacteria and inflammatory mediators enter the bloodstream. Vomiting, fever, and fluid shifting into the inflamed peritoneal space (third spacing) place the client at risk for hypovolemia (Option 4) from both fluid loss and reduced oral intake. Anemia (Option 1) is not an expected complication of appendiceal rupture, since this process does not typically cause significant blood loss. Dysrhythmias (Option 5) and cardiac arrest (Option 6) are late, severe complications that could theoretically follow untreated septic shock, but they are not among the 3 most directly expected complications of a ruptured appendix.

Question 4

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

1100:

Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

1130:

Notified primary health care provider about client status. Awaiting orders.

The nurse has reviewed the Nurses' Notes from 1130.

- For each potential intervention, click to specify whether the intervention is indicated or not indicated for the client.

| Potential Intervention                  | Indicated             | Not Indicated         |
|-----------------------------------------|-----------------------|-----------------------|
| clear liquid diet                       | <input type="radio"/> | <input type="radio"/> |
| soapsuds enema                          | <input type="radio"/> | <input type="radio"/> |
| heating pad to abdomen                  | <input type="radio"/> | <input type="radio"/> |
| abdominal girth measurements            | <input type="radio"/> | <input type="radio"/> |
| abdominal computed tomography (CT) scan | <input type="radio"/> | <input type="radio"/> |

Note: Each row must have only 1 response option selected.

Submit

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Question 4

COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Emergency Department

- 1100:

Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.
- 1130:

Notified primary health care provider about client status. Awaiting orders.

The nurse has reviewed the Nurses' Notes from 1130.

- For each potential intervention, click to specify whether the intervention is indicated or not indicated for the client.

| Potential Intervention                  | Indicated                        | Not Indicated                    |
|-----------------------------------------|----------------------------------|----------------------------------|
| clear liquid diet                       | <input type="radio"/>            | <input checked="" type="radio"/> |
| soapsuds enema                          | <input type="radio"/>            | <input checked="" type="radio"/> |
| heating pad to abdomen                  | <input type="radio"/>            | <input checked="" type="radio"/> |
| abdominal girth measurements            | <input type="radio"/>            | <input checked="" type="radio"/> |
| abdominal computed tomography (CT) scan | <input checked="" type="radio"/> | <input type="radio"/>            |

Note: Each row must have only 1 response option selected.

Submit

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CORRECT

Score: 5 / 5 points

Rationale:

A clear liquid diet is not indicated because the client should remain NPO (nothing by mouth) while an acute surgical abdomen is being evaluated and urgent surgery is anticipated; oral intake increases aspiration risk with anesthesia and can worsen nausea/vomiting. A soapsuds enema is not indicated because increasing intraluminal pressure and peristalsis in an inflamed or possibly obstructed/perforated bowel raises the risk of worsening perforation. A heating pad to the abdomen is not indicated because heat increases local blood flow and metabolic activity in inflamed tissue, which can accelerate an inflammatory process, and is contraindicated whenever appendicitis is suspected (it can increase the risk of rupture). Abdominal girth measurements are not indicated as a priority action here; they help trend progressive distension in conditions such as bowel obstruction or ascites, but they do not provide diagnostic information about the underlying disease process. An abdominal CT scan is indicated because it is the key diagnostic study needed to identify the cause of the client's pain — appendicitis, bowel obstruction, or a traumatic injury such as splenic rupture — and to guide the surgical plan.

Question 5

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Diagnostic Results

Emergency Department

1100:

Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

1130:

Notified primary health care provider about client status. Awaiting orders.

1230:

Client transported to radiology department for abdominal computed tomography (CT) scan.

1245:

20-gauge peripheral venous access device (VAD) inserted into the left hand. VAD site patent without signs of infiltration. 0.9% sodium chloride (normal saline) infusing at 75 mL/hr.

1400:

Client reports sudden relief of abdominal pain. Vital signs: T 102.5° F (39.2° C), P 110, RR 20, BP 125/86.

1415:

Primary health care provider notified about client status. Order received for an additional abdominal CT scan. Client transported to radiology department.

The nurse has reviewed the Nurses' Notes from 1230, 1245, 1400, and 1415 and the Diagnostic Results from 1230 and 1445.

➤ Complete the following sentence by choosing from the lists of options.

The nurse should insert 

Select...▼

. It would be a priority for the nurse to request a prescription for an 

Select...▼

. The nurse should prepare the client for surgery within 

Select...▼

.

Submit

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Question 5

COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Diagnostic Results

Emergency Department

1100:

Client reports nausea, loss of appetite, vomiting, fever, and constipation for the past 2 weeks and abdominal pain rated 7/10 on the Numerical Rating Scale for 1 week. Client states, "The abdominal pain started after my 7-year-old child accidentally kicked me in the stomach." Client plays soccer with the child once a week. Vital signs: T 103.4° F (39.7° C), P 92, RR 22, BP 130/86, pulse oximetry reading 98% on room air. No significant past medical or surgical history. Body mass index (BMI) of 32. Drinks alcohol only during social occasions, usually 3 beverages. Smokes cigarettes during social occasions.

1130:

Notified primary health care provider about client status. Awaiting orders.

1230:

Client transported to radiology department for abdominal computed tomography (CT) scan.

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1415:

Primary health care provider notified about client status. Order received for an additional abdominal CT scan. Client transported to radiology department.

The nurse has reviewed the Nurses' Notes from 1230, 1245, 1400, and 1415 and the Diagnostic Results from 1230 and 1445.

➤ Complete the following sentence by choosing from the lists of options.

The nurse should insert 

a nasogastric (NG) tube

 . It would be a priority for the nurse to request a prescription for an 

anti-infective medication

 . The nurse should prepare the client for surgery within 

6 hours

 .

Submit

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CORRECT

Score: 3 / 3 points

Rationale:

The CT scan confirms acute gangrenous appendicitis with a calcified appendicolith, and the follow-up scan — ordered after the client's sudden pain relief, which can signal that the appendix has ruptured and relieved pressure-related pain — confirms free intraperitoneal fluid consistent with a ruptured appendix. The nurse should insert a nasogastric (NG) tube, not a rectal tube or indwelling urethral catheter, to decompress the stomach, reduce nausea/vomiting, and reduce aspiration risk before surgery. An anti-infective (antibiotic) medication is the priority prescription to request, over an analgesic or antipyretic, because a ruptured appendix has caused peritoneal contamination and the client is at immediate risk for progression to sepsis/septic shock; treating the underlying infection takes priority, though analgesics and antipyretics may also ultimately be given for comfort. The client should be prepared for surgery within 6 hours, not 8 or 24 hours, because a ruptured appendix with free peritoneal fluid is a surgical emergency — delaying surgery further increases the risk of worsening sepsis and peritoneal contamination.

Question 6

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Diagnostic Results

Emergency Department

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1400:

Client reports sudden relief of abdominal pain. Vital signs: T 102.5° F (39.2° C), P 110, RR 20, BP 125/86.

1415:

Primary health care provider notified about client status. Order received for an additional abdominal CT scan. Client transported to radiology department.

1800:

Client transported to the operating room for an open appendectomy.

Medical-Surgical Unit

2030:

Client transported back to the medical-surgical unit.

2230:

Client performing coughing and deep-breathing exercises every hour while awake with the incentive spirometer. Performing postoperative leg exercises every hour while awake. Nasogastric (NG) tube removed. Drinking clear liquids. Abdomen boardlike with diminished bowel sounds in all quadrants. Rebound tenderness present.

The nurse has reviewed the Nurses' Notes from 1800, 2030, and 2230.

- Which of the following findings would indicate the client is progressing as expected? **Select all that apply.**
- ☐

1. clear, liquid diet
- ☐

2. board-like abdomen
- ☐

3. rebound tenderness
- ☐

4. incentive spirometry use
- ☐

5. diminished bowel sounds
- ☐

6. performance of leg exercises

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Question 6

COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department (ED) is caring for a 41-year-old male client.

Nurses' Notes

Diagnostic Results

Emergency Department

1100:

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The nurse has reviewed the Nurses' Notes from 1800, 2030, and 2230.

- Which of the following findings would indicate the client is progressing as expected? **Select all that apply.**
- ☐ 1. clear, liquid diet
- ☐ 2. board-like abdomen
- ☐ 3. rebound tenderness
- ☒ 4. incentive spirometry use
- ☐ 5. diminished bowel sounds
- ☒ 6. performance of leg exercises

Submit

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CORRECT

Score: 2 / 2 points

Rationale:

Incentive spirometry use (Option 4) and performance of leg exercises (Option 6) are expected, appropriate postoperative behaviors that indicate the client is progressing as expected: incentive spirometry and coughing/deep breathing reduce the risk of atelectasis and pneumonia, and leg exercises reduce the risk of venous thromboembolism during a period of reduced mobility after surgery. A clear liquid diet (Option 1) alone is not evidence of expected progress; how oral intake is advanced depends on the client's bowel and abdominal assessment findings, which in this case remain abnormal. A boardlike (rigid) abdomen (Option 2), rebound tenderness (Option 3), and diminished bowel sounds (Option 5) are concerning findings, not expected ones, after surgery for a ruptured appendix; a boardlike abdomen and rebound tenderness suggest ongoing peritoneal irritation or a new complication such as an abscess, and diminished bowel sounds can indicate a postoperative ileus. All 3 require follow-up rather than indicating expected progress.